

August 30, 2023

Initiative 23-0021

RECEIVED

Hon. Rob Bonta
Attorney General of California
1300 I Street, 17th Floor
Sacramento, CA 95814

Aug 30 2023

Attention: Ms. Anabel Renteria, Initiative Coordinator

INITIATIVE COORDINATOR
ATTORNEY GENERAL'S OFFICE

Re: Request for Title and Summary for Proposed Initiative Statute

Dear Mr. Bonta:

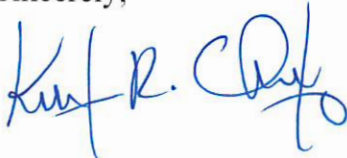
We serve as counsel for the proponents of the enclosed statewide initiative statute. On the proponent's behalf, I am enclosing the following documents:

- Proponent's request for a circulating title and summary.
- Proponent's certifications pursuant to Elections Code § 9001.
- Proponent's certifications pursuant to Elections Code § 9608.
- The text of the proposed initiative statute.
- A check in the amount of \$2,000.00

All legal inquiries or correspondence relative to this initiative should be directed to:

Kurt R. Oneto
Nielsen Merksamer LLP
1415 L Street, Suite 1200
Sacramento, CA 95814
(916) 446-6752
koneto@nmgovlaw.com

Sincerely,



Kurt R. Oneto
Enclosures

August 30

, 2023

VIA PERSONAL DELIVERY

Hon. Rob Bonta
Attorney General of California
1300 I Street, 17th Floor
Sacramento, CA 95814

Attention: Ms. Anabel Renteria, Initiative Coordinator

Re: Request for Title and Summary for Proposed Initiative Statute

Dear Mr. Bonta:

Pursuant to Article II, Section 10(d) of the California Constitution, I hereby submit the attached proposed Initiative Statute to your office and request preparation of a circulating title and summary of the chief points and purposes of the measure as provided by law. Included with this submission are the proponent affidavits signed by the proponent of this measure as required by Sections 9001 and 9608 of the California Elections Code, along with a check for \$2,000.00.

All inquires or correspondence relative to this initiative should be directed to Kurt R. Oneto at koneto@nmgovlaw.com or 916-446-6752.

Thank you for your assistance.

Sincerely,

A handwritten signature in blue ink, reading "Thomas Bannon", written over a horizontal line.

Thomas Bannon, Proponent

Enclosure: Proposed Initiative Statute

SECTION 1. *Article 3.3 (commencing with Section 14124.39) is added to Chapter 7 of Part 3 of Division 9 of the Welfare and Institutions Code, to read:*

ARTICLE 3.3. Protect Patients Now Act of 2024.

Section 14124.39. Title.

This article shall be known and may be cited as the Protect Patients Now Act of 2024.

Section 14124.40. Findings and declarations.

(a) In 1992, the federal government established a program giving safety net health care providers access to discounted prescription drugs. The intent of the law was for safety net health care providers to use the discounted drugs to treat patients who are “medically uninsured, on marginal incomes and have no other sources to turn to for preventive and primary care services” and to “reach[] more eligible patients and provide[] more comprehensive services” to “low-income and most vulnerable patients.” (H.R. Rep. No. 102-384 (Part 2), at 12 (1992)(Conf. Rep.)) The program was NOT intended to be used by safety net health care providers to accumulate massive fortunes running into the hundreds of millions of dollars or more.

(b) Unfortunately, some safety net health care providers have manipulated the program to receive enormous markups on the discounted prescription drugs they receive and then stick taxpayers with the added cost. Instead of using this massive windfall to help patients, the worst offenders have used their fortunes to purchase luxury coastal condominiums, wasted hundreds of millions of dollars on failed political campaigns, put elected politicians on their payrolls, and acquired low-income multifamily housing complexes that are operated as slums. Abusing revenues generated through the discount prescription drug program in this manner does not result in better health care for low-income patients. Instead, it cheats low-income patients out of the care they deserve and scams taxpayers who end up footing the bill.

(c) Governor Newsom has already ended this type of prescription drug scamming in the Medi-Cal program through Executive Order N-01-19, which requires the Department of Health Care Services to transition Medi-Cal pharmacy services away from arrangements that are susceptible to price gouging. Known as the Medi-Cal Rx program, it achieves cost-savings for

prescription drug purchases made by the State, standardizes the pharmacy benefit statewide for all Medi-Cal patients, increases overall access, and eliminates the ability of prescription drug price manipulators to game the system through Medi-Cal. However, other aspects of the discount prescription drug program that price manipulators still exploit have not yet been addressed.

(d) California needs to make the cost-savings achieved through the Medi-Cal Rx program permanent. Furthermore, additional reforms are necessary to protect taxpayer dollars and help the neediest patients by ensuring that prescription drug price manipulators are required to end all of their remaining price-gouging schemes in order to continue operating in our State.

Section 14124.41. Statement of intent.

In enacting this article, the purpose and intent of the People of the State of California is to do all of the following:

(a) To make permanent the Medi-Cal Rx program so that its expanded patient access and cost-savings can be continued in perpetuity.

(b) To protect patients and taxpayers by putting an end to other price-gouging schemes that are still being perpetrated in our State through the discount prescription drug program.

(c) To impose strict accountability on prescription drug price manipulators by requiring them to spend at least ninety-eight percent (98%) of their revenues generated through the discount prescription drug program on direct patient care.

(d) To ensure that health care providers that have a track record of running price-gouging schemes through the discount prescription drug program refocus on providing direct patient care or lose their state-provided privileges and benefits including suspension and revocation of licenses, loss of state and local grant funding, and elimination of tax-exempt status.

Section 14124.42. Permanent authorization for the Medi-Cal Rx program.

The Department of Health Care Services is authorized to provide and administer Medi-Cal pharmacy services under a single statewide fee-for-service delivery system.

Section 14124.43. Prescription drug price-gouging schemes prohibited.

(a) On and after January 1, 2025, a prescription drug price manipulator shall not enter into, or participate in, a prescription drug price-gouging scheme that applies to, operates in, or intends or proposes to operate in or apply to, this State.

(b) Any contract or other agreement that qualifies a prescription drug price-gouging scheme is, as of January 1, 2025, contrary to public policy and is void and unenforceable to the extent that the prescription drug price-gouging scheme applies to, operates in, or intends or proposes to operate in or apply to, this State.

Section 14124.44. Patient protection requirements imposed on prescription drug price manipulators.

Notwithstanding any other provision of law, on and after January 1, 2025, a prescription drug price manipulator shall only be eligible for tax-exempt status in this State or to be licensed to operate as a pharmacy, a health care service plan, or a clinic in this State if it complies with all of the following requirements:

(a) In the prior calendar year, the prescription drug price manipulator spent at least ninety-eight percent (98%) of the revenues it generated in California from participation in the discount prescription drug program on direct patient care.

(b) In the prior calendar year, the prescription drug price manipulator was not engaged in any unprofessional conduct, dishonest dealing, or conduct inimical to the public health, welfare, or safety of the people of the State of California.

(c) In the prior calendar year, the prescription drug price manipulator was not a party to, and did not participate in, a prescription drug price-gouging scheme that operated in, or applied to, this State.

Section 14124.45. Oversight of prescription drug price manipulators.

(a) In order to determine compliance with Section 14124.44, on and after January 1, 2025:

(1) A prescription drug price manipulator that holds tax-exempt status in this State shall annually submit to the Attorney General a detailed accounting for the prior calendar year of both

its California statewide and nationwide revenues generated from participation in the discount prescription drug program as well as how those revenues were spent.

(2) A prescription drug price manipulator that holds a pharmacy license shall annually submit to the Board of Pharmacy a detailed accounting for the prior calendar year of both its California statewide and nationwide revenues generated from participation in the discount prescription drug program as well as how those revenues were spent.

(3) A prescription drug price manipulator that holds a health care service plan license shall annually submit to the Department of Managed Health Care a detailed accounting for the prior calendar year of both its California statewide and nationwide revenues generated from participation in the discount prescription drug program as well as how those revenues were spent.

(4) A prescription drug price manipulator that holds a clinic license shall annually submit to the Department of Public Health a detailed accounting for the prior calendar year of both its California statewide and nationwide revenues generated from participation in the discount prescription drug program as well as how those revenues were spent.

(b) The Attorney General, the Board of Pharmacy, the Department of Managed Health Care, or the Department of Public Health may request from the prescription drug price manipulator any other information deemed necessary or convenient to determine compliance with the requirements set forth in Section 14124.44.

(c) All information submitted pursuant to this section shall be submitted under penalty of perjury.

(d)(1) All financial information submitted to the Attorney General, Board of Pharmacy, Department of Managed Health Care, or Department of Public Health pertaining to either of the following shall be treated as confidential and sensitive business information exempt from public disclosure:

(A) Specific prices or amounts paid by, or charged to, a prescription drug price manipulator for prescription drugs acquired by the prescription drug price manipulator through the discount prescription drug program.

(B) Specific prices or amounts charged by, or paid to, a prescription drug price manipulator for prescription drugs it obtained through the discount prescription drug program.

(2) Total aggregated revenues generated by a prescription drug price manipulator through the discount prescription drug program are exempt from this subdivision so long as the figures do not reveal the information described in subparagraphs (A) or (B) of paragraph (1).

(e)(1) The Attorney General, Board of Pharmacy, Department of Managed Health Care, and Department of Public Health shall cooperatively establish the deadline each year for a prescription drug price manipulator to submit the information required by this section.

(2) For calendar year 2025, the deadline shall not be later than September 1, 2025.

(3) A prescription drug price manipulator that fails to submit required information by the deadline established pursuant to this subdivision shall be deemed to be out of compliance with the requirements of Section 14124.44 for the applicable calendar year, according to the procedures set forth in subdivision (b) of Section 14124.46.

(f) The Attorney General, Board of Pharmacy, Department of Managed Health Care, and Department of Public Health may each impose a fee on a prescription drug price manipulator for the costs associated with concluding whether the prescription drug price manipulator was in compliance with the requirements of Section 14124.44 during the prior calendar year. The charges shall not exceed the reasonable regulatory costs to the respective agency incident to performing the investigations, inspections, and audits required by this article, including any administrative enforcement and adjudication thereof.

Section 14124.46. Conclusions regarding compliance.

(a)(1) Within 45 calendar days of the deadline established pursuant to subdivision (e) of Section 14124.45, the Attorney General, Board of Pharmacy, Department of Managed Health Care, and Department of Public Health shall each separately issue an independent written conclusion regarding whether or not the prescription drug price manipulator is in compliance with the requirements of Section 14124.44.

(2) The Attorney General, Board of Pharmacy, Department of Managed Health Care, or Department of Public Health shall only be required to issue an independent written conclusion pursuant to this subdivision if the prescription drug price manipulator was required to submit information to the relevant official, board, or department pursuant to subdivision (a) of Section 14124.45.

(b)(1) If, within the 45-day calendar period set forth in paragraph (1) of subdivision (a), the information submitted by a prescription drug price manipulator is found to be incomplete or insufficient by the Attorney General, Board of Pharmacy, Department of Managed Health Care, or Department of Public Health for issuance of a written conclusion required by this section, then the relevant official, board, or department shall issue to the prescription drug price manipulator a written notice to correct. The notice to correct shall contain a description of the additional information required.

(2) The prescription drug price manipulator shall have 10 calendar days from the date of the notice to correct to provide complete and/or sufficient information. If the prescription drug price manipulator fails to remedy the incompleteness and/or insufficiency within 10 calendar days, then the prescription drug price manipulator shall be deemed to be out of compliance with the requirements of Section 14124.44 for the applicable calendar year and the relevant official, board, or department shall issue a written conclusion to that effect immediately upon the expiration of the 10 calendar day period.

(c) The Attorney General, Board of Pharmacy, Department of Managed Health Care, or Department of Public Health may, either collectively or separately, enter into an inter-agency agreement with the California State Auditor's Office for assistance in reaching a conclusion about a prescription drug price manipulator's compliance with the requirements of Section 14124.44. Costs incurred pursuant to an inter-agency agreement under this subdivision may be recovered pursuant to subdivision (f) of Section 14124.45.

(d)(1) If the Attorney General concludes a prescription drug price manipulator is not in compliance with the requirements of Section 14124.44, then any exemption from state taxation held by the prescription drug price manipulator shall be suspended as of that date. The tax exempt status shall remain suspended pending a final determination pursuant to this article.

(2) If either the Attorney General or Board of Pharmacy concludes a prescription drug price manipulator is not in compliance with the requirements of Section 14124.44, then any and all pharmacy licenses held by the prescription drug price manipulator shall be suspended as of that date. The license(s) shall remain suspended pending a final determination pursuant to this article.

(3) If either the Attorney General or Department of Managed Health Care concludes a prescription drug price manipulator is not in compliance with the requirements of Section

14124.44, then any and all health care service plan licenses held by the prescription drug price manipulator shall be suspended as of that date. The license(s) shall remain suspended pending a final determination pursuant to this article.

(4) If either the Attorney General or Department of Public Health concludes a prescription drug price manipulator is not in compliance with the requirements of Section 14124.44, then any and all clinic licenses held by the prescription drug price manipulator shall be suspended as of that date. The license(s) shall remain suspended pending a final determination pursuant to this article.

Section 14124.47. Final determinations.

(a) A written conclusion issued by the Attorney General, Board of Pharmacy, Department of Managed Health Care, or Department of Public Health regarding a prescription drug price manipulator's compliance with the requirements of Section 14124.44 shall be a final determination, except that any party aggrieved by the conclusion may seek review pursuant to Section 1094.5 of the Code of Civil Procedure within 30 calendar days of issuance of the conclusion. If review is sought pursuant to Section 1094.5 of the Code of Civil Procedure, the final determination shall be based upon the outcome of that review.

(b) If, pursuant to the procedures set forth in this article, a prescription drug price manipulator is finally determined to have violated the requirements of Section 14124.44, then all of the following shall apply:

(1) Any and all pharmacy licenses, health care service plan licenses, or clinic licenses held by the prescription drug price manipulator shall be permanently revoked.

(2) The prescription drug price manipulator shall be prohibited from applying for, or obtaining or possessing, a pharmacy license, health care service plan license, or clinic license for a period of 20 years.

(3) Any person serving as an owner, officer, or director of the prescription drug price manipulator at the time the violation of Section 14124.44 occurred shall be prohibited from serving as an owner, officer, or director of a licensed pharmacy, health care service plan, or clinic for a period of 20 years.

(4) The prescription drug price manipulator shall lose, and no longer be eligible for, tax-exempt status in the State of California, including under Chapter 4 (commencing with Section

23701) of Part 11 of Division 2 of the Revenue and Taxation Code, and shall instead be subject to the Revenue and Taxation Code as a taxable organization. The prescription drug price manipulator shall be prohibited from reapplying for, or again being granted, tax-exempt status for a period of 20 years.

(5) The prescription drug price manipulator shall be ineligible to receive any new or renewed state or local grants or contracts for a period of 20 years.

Section 14124.48. Definitions.

For purposes of this article, as used in both the singular and plural form, the following definitions shall apply:

(a) “Clinic” means an entity operating as one or more of the clinics described in Section 1204 of the Health and Safety Code.

(b) “Direct patient care” means the provision of medical services, dental services, pharmaceutical services, or behavioral health services directly administered to individual patients being treated for, or suspected of having, physical or mental illnesses. Direct patient care includes preventive care that is directly administered to patients. Further, in order to qualify as “direct patient care,” the services must be health care services that are regularly provided by other health care providers in the community or nonprofit community-based organizations who are also receiving reimbursements or payments from either the Medi-Cal or Medicare programs.

(c) “Enforcement agency” means any department of a state, county, or city agency that has the authority to inspect a multifamily dwelling and enforce health, safety, or building codes including, but not limited to, the building department or building division, the housing department, the housing and community investment department, the fire department or fire district, and the health department.

(d) “Entity” means a natural person, corporation, or other legal or corporate organization of any kind, whether non-profit or for-profit, and includes any parent, subsidiary, or affiliate of the entity.

(e) “Health care service plan” means an entity operating as a health care service plan under the Knox-Keene Health Care Service Plan Act of 1975 (Chapter 2.2 (commencing with Section 1340) of Division 2 of the Health and Safety Code).

(f) “Discount prescription drug program” means the program established by the Veterans Health Care Act of 1992, P.L. 102-585 § 602, within section 340B of the Public Health Service Act (§ 340B; 42 U.S.C § 256b) that is administered by the Office of Pharmacy Affairs in the Health Resources and Services Administration within the United States Department of Health and Human Services.

(g) “Medi-Cal Rx Program” means the program initially established pursuant to Executive Order N-01-19 and permanently authorized by Section 14124.42.

(h) “Multifamily dwelling” means any structure located in this State designed for human habitation or occupancy that has been divided into two or more independent living quarters.

(i) “Owner-operator of highly dangerous properties” means an entity, including any parent, subsidiary, or affiliate of that entity, that owns, operates, or is the responsible party for one or more multifamily dwellings that meet the following conditions:

(1) One or more of the multifamily dwellings have been inspected on one or more occasions by an enforcement agency or officer thereof.

(2) The enforcement agencies or officers issued one or more written notices or inspection reports identifying violations affecting the health and safety of occupants of the multifamily dwelling(s).

(3) Cumulatively across all of the multifamily dwellings, the written notices and/or inspection reports described in paragraph (2) have identified a combined total of at least five hundred (500) violations which were categorized in violation severity level “high.”

(j) “Pharmacy” means an entity operating pursuant to Chapter 9 (commencing with Section 4000) of Division 2 of the Business and Professions Code.

(k)(1) “Prescription drug price-gouging scheme” means any agreement involving a prescription drug price manipulator and a pharmacy whereby:

(A) The pharmacy dispenses drugs obtained by the prescription drug price manipulator through the discount prescription drug program.

(B) The price charged by the pharmacy for the drugs described in subparagraph (A) exceeds the purchase price negotiated and/or paid by the prescription drug price manipulator pursuant to or through the discount prescription drug program.

(2) A prescription drug-price gouging scheme can exist between unrelated entities, or between related entities that are parents, subsidiaries, or affiliates of one another or otherwise under common ownership or control.

(l) “Prescription drug price manipulator” means an entity, including any parent, subsidiary, or affiliate of that entity, that individually or collectively with one or more of its parents, subsidiaries, or affiliates meets all of the following requirements:

(1) The entity purchases, authorizes, or obtains prescription drugs through the discount prescription drug program.

(2) During any ten calendar year period of its existence, the entity spent more than one hundred million dollars (\$100,000,000) on purposes that do not qualify as direct patient care.

(3) The entity currently is, or has previously been, an owner-operator of highly dangerous properties.

(4) The entity meets at least one of the following conditions:

(A) The entity currently has, or previously had, one or more licenses to operate as a health care service plan.

(B) The entity currently contracts, or has previously contracted, with the Department of Health Care Services as a primary care case management organization pursuant to Article 2.9 (commencing with Section 14088) of Chapter 7 of Part 3 of Division 9.

(C) The entity currently contracts, or has previously contracted, with the federal Centers for Medicare and Medicaid Services to provide services in the Medicare Program as a Medicare special needs plan.

(D) The entity currently has, or previously had, one or more licenses to operate as a pharmacy.

(E) The entity currently has, or previously had, one or more licenses to operate as a clinic.

14124.49. Unprofessional conduct, dishonest dealing, and conduct inimical to public health, welfare, or safety.

(a) In addition to any other conduct, standard, or requirement described in Article 7 (commencing with Section 1386) of Chapter 2.2 of Division 2 of the Health and Safety Code or any other statute or regulation, it shall constitute dishonest dealing for a health care service plan

that qualifies as a prescription drug price manipulator to spend, in any calendar year, less than ninety-eight percent (98%) of all the revenues it generates from participation in the discount prescription drug program on direct patient care.

(b) In addition to any other conduct, standard, or requirement described in Article 19 (commencing with Section 4300) of Chapter 9 of Division 2 of the Business and Professions Code, Section 1762 of Title 16 of the California Code of Regulations, or any other statute or regulation, it shall constitute unprofessional conduct for a pharmacy that qualifies as a prescription drug price manipulator to spend, in any calendar year, less than ninety-eight percent (98%) of all the revenues it generates from participation in the discount prescription drug program on direct patient care.

(c) In addition to any other conduct, standard, or requirement described Article 5 (commencing with Section 1240) of Chapter 1 of Division 2 of the Health and Safety Code or any other statute or regulation, it shall constitute conduct inimical to the public health, welfare, or safety of the people of the State of California for a clinic that qualifies as a prescription drug price manipulator to spend, in any calendar year, less than ninety-eight percent (98%) of all the revenues it generates from participation in the discount prescription drug program on direct patient care.

Section 14124.50. Public input.

The Attorney General, Board of Pharmacy, Department of Managed Health Care, and Department of Public Health shall invite, and provide a process for submission of, public comments and information relating to entities that qualify as a prescription drug price manipulator and/or an owner-operator of highly dangerous properties. Information that can be submitted pursuant to this section includes, but is not limited to, records of expenditures and written notices or inspection reports identifying violations affecting the health and safety of occupants at multifamily dwelling(s).

Section 14124.51. Effective date and severability.

(a) This article shall take effect on the next January 1 following its adoption by the voters.

(b) The provisions of this article are severable. If any portion, section, subdivision, paragraph, clause, subclause, sentence, phrase, word, or application of this article is for any reason held to be invalid by a decision of any court of competent jurisdiction, that decision shall not affect the validity of the remaining portions of this article. The People of the State of California hereby declare that they would have adopted this article and each and every portion, section, subdivision, paragraph, clause, subclause, sentence, phrase, word, and application not declared invalid or unconstitutional without regard to whether any part of this article or application thereof would be subsequently declared invalid.

SECTION 2. *Conflicting Measures.*

(a) In the event that this initiative measure and another initiative measure or measures dealing with prescription drug price-gouging schemes or prescription drug price manipulators, as defined in this initiative, shall appear on the same statewide election ballot, the other initiative measure or measures shall be deemed to be in conflict with this measure. In the event that this initiative measure receives a greater number of affirmative votes, the provisions of this measure shall prevail in their entirety, and the provisions of the other initiative measure or measures shall be null and void.

(b) Notwithstanding subdivision (a), the People hereby find and declare that this initiative measure does *not* conflict with any other initiative measure that provides additional or extended funding for the Medi-Cal program.

SECTION 3. *Liberal Construction.*

This Act shall be liberally construed to give effect to its intent and purposes, which are expressed in Sections 14124.40 and 14124.41.

SECTION 4. *Legal Defense.*

The purpose of this section is to ensure that the people's precious right of initiative cannot be improperly annulled by state politicians who refuse to defend the will of the voters. Therefore, if this Act is approved by the voters of the State of California and thereafter subjected to a legal challenge which attempts to limit the scope or application of this Act in any way, or

alleges this Act violates any state or federal law in whole or in part, and both the Governor and Attorney General refuse to defend this Act to the fullest extent possible on behalf of the State of California, then the following actions shall be taken:

(a) Notwithstanding anything to the contrary contained in Chapter 6 (commencing with Section 12500) of Part 2 of Division 3 of Title 2 of the Government Code or any other law, the Attorney General shall appoint independent counsel to faithfully and vigorously defend this Act to the fullest extent possible on behalf of the State of California.

(b) Before appointing or thereafter substituting independent counsel, the Attorney General shall exercise due diligence in determining the qualifications of independent counsel and shall obtain written affirmation from independent counsel that independent counsel will faithfully and vigorously defend this Act to the fullest extent possible. The written affirmation shall be made publicly available upon request.

(c) In order to support the defense of this Act in instances where the Governor and Attorney General fail to do so despite the will of the voters, a continuous appropriation is hereby made from the General Fund to the Controller, without regard to fiscal years, in an amount necessary to cover the costs of retaining independent counsel to faithfully and vigorously defend this Act on behalf of the State of California to the fullest extent possible.